INFORMATION TO KEEP WITH YOU

# My complete treatment handbook

## Nivolumab, gemcitabine and carboplatin before surgery

Understand your treatment. Know what to expect. Keep your plan to hand.

**Displayed calendar**
**21 days**
See the active treatment phase

**Treatment stages**
**2**
Explained in the treatment reference

**Regimen source**
**NCCP 00849**
Version 2

Up to three 21-day cycles. Day 1: nivolumab, gemcitabine and carboplatin. Day 8: gemcitabine alone after blood tests and review. Then surgical multidisciplinary team (MDT) reassessment. The five listed backbones are alternatives; this guide covers only your selected combination.

### Treatment stages

01 **Before surgery**
Up to three cycles

02 **Surgical reassessment**
After treatment

Your team confirms the active stage and dates. The full treatment guide explains each step.

### Inside your handbook


| | | PAGE |
|---|---|---|
| 01 | Understanding your treatment | 2 |
| 02 | Your schedule, tests and appointments | 5 |
| 03 | Symptoms and supportive care | 9 |
| 04 | Your record and contact card | 14 |
| 05 | Sources and further information | 19 |


**My name**

**Hospital / oncology team**

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

# Understanding your treatment

Nivolumab, gemcitabine and carboplatin before surgery · National Cancer Control Programme (NCCP) 00849

## When to get help

**Emergency: call 112 / 999**

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

**Contact your oncology service immediately**

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact your oncology team urgently for fever, feeling suddenly unwell, new breathlessness, chest pain, persistent diarrhoea, severe weakness, confusion, reduced urine or a severe rash. Follow your local emergency instructions; do not wait for the next appointment.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

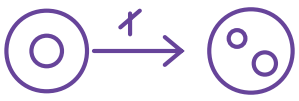
## Three medicines. Different jobs.

This treatment is given before planned surgery for resectable non-small-cell lung cancer at high risk of returning. It aims to treat the cancer before the operation and reduce the chance of recurrence. Surgery and benefit are not guaranteed.

IMMUNOTHERAPY

### Nivolumab

Opdivo



**Blocks the PD-1 immune checkpoint**

Nivolumab binds PD-1 on T cells. It prevents signals that can reduce the immune attack on cancer. This can also let immune cells inflame healthy tissues. It is infused every 21 days in this plan.

T cell → PD-1 blocked → Immune response

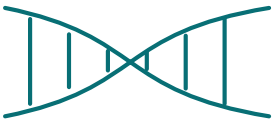
**What to watch for**

New breathing, bowel, skin, hormone or other symptoms need prompt advice during or after treatment.

CHEMOTHERAPY

### Gemcitabine

Antimetabolite chemotherapy



**Interferes with deoxyribonucleic acid (the cell's genetic material) (DNA) building**

Gemcitabine interferes with making DNA, which dividing cancer cells need. Healthy blood-forming cells and other tissues can also be affected.

DNA building → Replication disrupted → Dividing cells affected

**What to watch for**

Report fever, unusual bleeding, breathlessness, rash, swelling or reduced urine promptly.

CHEMOTHERAPY

### Carboplatin

Platinum chemotherapy



**Damages DNA**

Carboplatin forms links in DNA that hinder copying and repair. Healthy tissues can also be harmed. Its dose depends on target exposure (AUC) and measured or estimated kidney function; AUC is not a fixed number of milligrams. There are up to three planned doses, one in each neoadjuvant cycle before surgical review.

DNA → Platinum links → Copying disrupted

**What to watch for**

Report fever, bleeding, nausea, hearing changes or numbness. Tell the nurse immediately about possible allergy during the infusion.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

## Your treatment before surgery

### Why these medicines?

Nivolumab helps immune cells recognise cancer; chemotherapy damages dividing cells. Your prescribed backbone is gemcitabine and carboplatin.

**Evidence and uncertainty**

In a randomized study, adding nivolumab to chemotherapy before surgery improved survival compared with chemotherapy alone. Your team considers the exact cancer, test results, fitness and preferences.

**Alternatives**

Discuss surgery and other suitable treatments, clinical trials and the option of not proceeding; choices depend on the multidisciplinary assessment.

## Why treatment is offered before surgery

The trial population was broader than NCCP eligibility. Trial averages cannot predict your own outcome or compare these five chemotherapy options.

SACTCHECK · CHAPTER 02

# Your schedule, tests and appointments

Nivolumab, gemcitabine and carboplatin before surgery · NCCP 00849

## Induction → review → maintenance

### Treatment stages

01 **Before surgery**
Up to three cycles

02 **Surgical reassessment**
After treatment

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

UP TO THREE CYCLES

### Before surgery

1 2 3

Up to three 21-day cycles. Day 1: nivolumab, gemcitabine and carboplatin. Day 8: gemcitabine alone after blood tests and review. Then surgical multidisciplinary team (MDT) reassessment. The five listed backbones are alternatives; this guide covers only your selected combination.

AFTER TREATMENT

### Surgical reassessment

MDT review

The surgical team reviews scans, fitness, recovery and timing. This regimen does not automatically continue nivolumab after surgery.

Maintenance means ongoing treatment, not treatment without side effects. Your plan can change after discussion.

BASELINE → TREATMENT → FOLLOW-UP

## Tests and decisions across the treatment course

These are relative stages, not equal time intervals. Additional tests depend on symptoms and the selected chemotherapy source.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00849 v2 · official regimen · NCCP 00310 v8 · selected chemotherapy

THE WHOLE TREATMENT JOURNEY

# Nivolumab, gemcitabine and carboplatin before surgery: treatment and checks

**Neoadjuvant non-small-cell lung cancer (NSCLC) · selected gemcitabine / carboplatin backbone**

Up to three 21-day cycles. Day 1: nivolumab, gemcitabine and carboplatin. Day 8: gemcitabine alone after blood tests and review. Then surgical MDT reassessment. The five listed backbones are alternatives; this guide covers only your selected combination.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Preparation | Cycle 1 | Cycle 2 | Cycle 3 | Surgical review | Follow-up |
|---|---|---|---|---|---|---|
| **Treatment**<br>NCCP 00849 · COMPANION | Confirm selected backbone | Day 1 all three; Day 8 gemcitabine | Repeat if appropriate | Final planned cycle | No automatic systemic anticancer therapy (SACT) continuation | Separate plan if further treatment needed |
| **Blood and organ tests**<br>NCCP 00849 · COMPANION | full blood count (FBC), kidney, liver, glucose, thyroid and viral screen | FBC, kidney, liver, glucose; Day 8 counts | Repeat before treatment | Repeat before treatment | Recovery and fitness checks | Follow unresolved abnormalities |
| **Immune monitoring**<br>NCCP 00849 · COMPANION | Document baseline symptoms | Symptoms before every dose | Thyroid every 3–6 weeks | Cortisol if clinically indicated | Report delayed symptoms | Vigilance at least five months |

| Care / check | Preparation | Cycle 1 | Cycle 2 | Cycle 3 | Surgical review | Follow-up |
|---|---|---|---|---|---|---|
| **Support and decisions**<br>NCCP 00849 · COMPANION | Follow the written anti-sickness plan; Day 8 gemcitabine has its own review and support instructions. Carboplatin dosing uses a verified kidney-function measurement; report dehydration or a change in health. | Confirm written prevention plan | Review tolerability | Plan end of chemotherapy support | MDT agrees imaging and timing | Agree contacts and surveillance |

## What each check is for

### 1 Preparation

Before first infusion

Confirm the exact indication and backbone

**Ask** / **act:** Complete pathology, molecular, organ-function and consent review.

### 2 Each cycle

Before each planned dose

Treatment safety can change

**Ask** / **act:** Review toxicity, results and the active support plan.

### 3



### Surgery assessment

After at most three cycles

Coordinate recovery and local treatment

**Ask** / **act:** Agree surgery timing; no automatic postoperative nivolumab.

### 4 Follow-up

During and after surgery

Immune effects may be delayed

**Ask** / **act:** Keep the immune alert card and report new symptoms promptly.

**At the next review, ask**

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

## Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

### Before the visit

Confirm blood tests, transport, medicine instructions and the oncology contact number.

STEP 2

### Time in the unit

Nivolumab is infused over 30 minutes before chemotherapy. Gemcitabine is infused over 30 minutes on Days 1 and 8. Carboplatin is infused over 30 minutes. Blood-result review, preparation, support medicines and observation add to the visit.

STEP 3

### Access

Treatment uses suitable intravenous (IV) access. No routine take-home infusion pump is used.

STEP 4

### Home support

Follow the written anti-sickness plan; Day 8 gemcitabine has its own review and support instructions. Carboplatin dosing uses a verified kidney-function measurement; report dehydration or a change in health.

STEP 5

### Surgical review

Keep the oncology and surgical teams informed about new symptoms and other appointments.

## Treatment visits

**Day-by-day checks**

Blood count, kidneys, liver and glucose before each cycle; thyroid tests every 3–6 weeks; cortisol if indicated. Day 8 blood count before gemcitabine. Assess symptoms before every dose.

# Symptoms and supportive care

Nivolumab, gemcitabine and carboplatin before surgery · NCCP 00849

## Your 21-day cycle.

Timing is a general guide; symptoms may start or stop at other times.

| Treatment or check | Timing in this view |
|---|---|
| **All three medicines** |  **Day 1** After clinical review |
| **Gemcitabine alone** |  **Day 8** Separate blood tests and review |
| **Immune symptoms can occur any day** |  **Days 1–21** Nivolumab can inflame healthy organs during treatment or after it stops. Report new symptoms promptly and carry your immune alert card for at least five months after the last dose. |

Relative cycle days, not guaranteed symptom or recovery dates.

The next cycle starts 21 days after this one, if your team confirms it.

> Contact your oncology team urgently for fever, feeling suddenly unwell, new breathlessness, chest pain, persistent diarrhoea, severe weakness, confusion, reduced urine or a severe rash. Follow your local emergency instructions; do not wait for the next appointment.

### Between-treatment notes

**Day 1: Day 1 treatment**

All three medicines after review. Confirm symptoms, blood results and support plan.

**Day 2–7: At home**

Follow the written support plan. Report new symptoms promptly.

**Day 8: Day 8 treatment**

Gemcitabine alone if blood tests and review allow. Do not assume Day 1 clearance covers Day 8.

**Day 9–21: Recovery and review**

Effects vary; no guaranteed recovery day. Keep the contact plan accessible.

## Support around the medicines

### 1 Immune alert card

Nivolumab can inflame healthy organs during treatment or after it stops. Report new symptoms promptly and carry your immune alert card for at least five months after the last dose.

### 2 Prepare for surgery

The oncology and surgical teams agree scans, recovery checks and timing. Report any illness or new symptom before the planned operation.

## Recognise effects early

### During an infusion

Tell the nurse immediately about chills, rash, flushing, chest discomfort, breathing difficulty or feeling faint.

## Everyday care

Plan support around how you feel.

### Eating and drinking

Ask for help with nausea, appetite, mouth pain or poor intake. Follow any individual fluid restriction or hydration plan.

### Medicines and pregnancy

Tell your team about all medicines and supplements. Discuss fertility and contraception before treatment; pregnancy and breastfeeding require specific advice.

## Moving towards surgery

The surgical team decides readiness.

### Reassessment

Scans, clinical recovery, organ function and fitness inform the operation plan. Treatment response does not remove the need for surgical review.

## Your contacts and questions

### Keep your records

Bring the treatment passport, medicines list and immune alert card to appointments or emergency care.

### Understanding the plan

Ask your team to confirm the selected backbone, next visit, support medicines and whom to call.

## Questions worth asking

### Will I receive all five combinations?

No. This guide is for gemcitabine and carboplatin with nivolumab. The five backbones are alternatives; your team prescribes one.

### Does nivolumab continue after surgery?

Not automatically under this NCCP regimen. It covers at most three cycles before surgery; any later treatment needs a separate plan.

### What if a dose is delayed?

The team reviews blood tests, symptoms and recovery. Do not assume a delay means treatment has failed or that surgery must occur on a fixed date.

**Try explaining the plan back:** Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

### Nivolumab

- Immune inflammation in multiple organs
- Infusion reactions
- Delayed effects after treatment stops

### Gemcitabine

- Low blood counts and infection
- Nausea, fatigue and flu-like symptoms
- Skin rash and swelling
- Rare serious lung or kidney injury

### Carboplatin

- Nausea and vomiting
- Kidney, hearing and nerve effects
- Allergic reactions

**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Day 1 · gemcitabine / carboplatin with nivolumab

**Active medicines:** nivolumab + gemcitabine + carboplatin

**Emetogenic risk:** high

| Medicine | Dose / route | When |
|---|---|---|
| Aprepitant | 125 mg orally Day 1; 80 mg orally Days 2–3 | Before systemic anticancer therapy (SACT) on Day 1, then daily on Days 2–3 |
| Ondansetron | 16 mg orally | Day 1<br>Adjust for QT/individual risk. |

| Medicine | Dose / route | When |
|---|---|---|
| Antiemetic dexamethasone reference | 12 mg orally Day 1; 8 mg orally daily Days 2–4 | Dedicated carboplatin AUC ≥4 NCIS V8 row<br>Reference course only: reconcile the aprepitant interaction and any other steroids into one clear prescription. |
| Metoclopramide | 10 mg orally three times daily as prescribed | Subsequent days<br>NCIS row does not specify an end day or PRN qualifier; prescriber/pharmacy must specify duration and suitability. |

### Additional phase-specific support

- Follow the written anti-sickness plan; Day 8 gemcitabine has its own review and support instructions. Carboplatin dosing uses a verified kidney-function measurement; report dehydration or a change in health.
- Nivolumab can inflame healthy organs during treatment or after it stops. Report new symptoms promptly and carry your immune alert card for at least five months after the last dose.
- The prescriber specifies doses, duration, rescue treatment and interactions; follow one reconciled prescription.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

## Day 8 · gemcitabine alone

**Active medicines:** gemcitabine

**Emetogenic risk:** low

| Medicine | Dose / route | When |
|---|---|---|
| Dexamethasone | 8 mg orally | Day 8 before gemcitabine if prescribed<br>NCIS low-risk reference; individualise and reconcile with any other steroid course. |

### Additional phase-specific support

- Separate Day 8 blood-count and symptom review. Do not repeat Day 1 high-risk prophylaxis automatically.
- Nivolumab can inflame healthy organs during treatment or after it stops. Report new symptoms promptly and carry your immune alert card for at least five months after the last dose.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00849 · VERSION 2

# Nivolumab, gemcitabine and carboplatin before surgery

Up to three 21-day cycles. Day 1: nivolumab, gemcitabine and carboplatin. Day 8: gemcitabine alone after blood tests and review. Then surgical MDT reassessment. The five listed backbones are alternatives; this guide covers only your selected combination.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

## A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

### 1 Hormones / energy

**Nivolumab**

Report severe tiredness, headache, dizziness or unusual thirst.

### 2 Breathing

**Nivolumab / gemcitabine**

Report new cough, breathlessness or chest pain urgently.

### 3 Liver

**Nivolumab / chemotherapy**

Blood tests check liver function; report yellow eyes or dark urine.

### 4 Bowels

**Nivolumab / chemotherapy**

Report persistent diarrhoea, blood, pain or inability to drink.

### 5 Kidneys / urine

**Nivolumab / carboplatin**

Blood tests check kidney function; report reduced urine.

### 6 Blood / infection

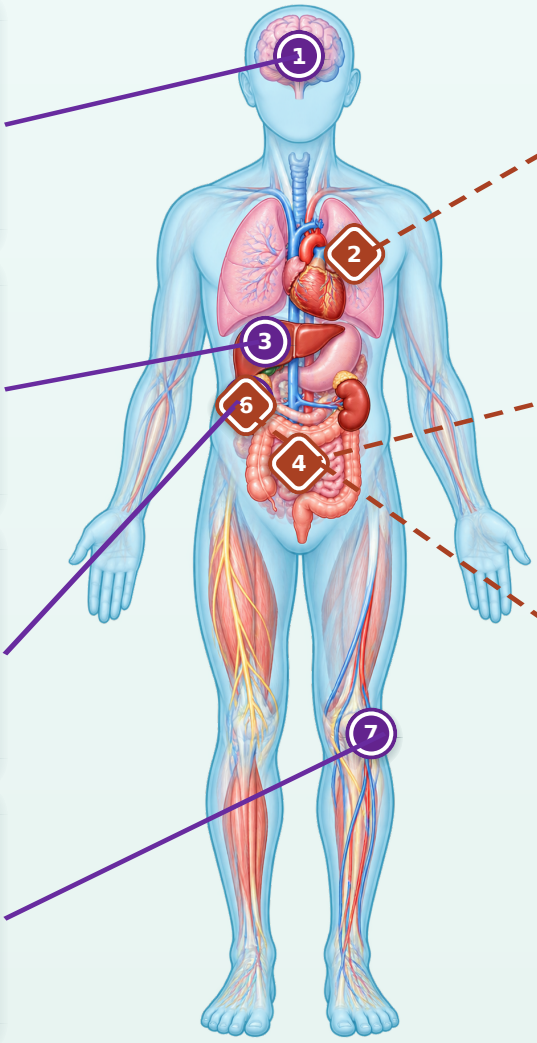
**gemcitabine / carboplatin**

Fever or sudden illness can be an emergency; follow your contact plan.

### 7 Skin / hair

**gemcitabine / nivolumab**

Report a severe rash or blistering urgently; hair changes vary.




Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact your oncology team urgently for fever, feeling suddenly unwell, new breathlessness, chest pain, persistent diarrhoea, severe weakness, confusion, reduced urine or a severe rash. Follow your local emergency instructions; do not wait for the next appointment.

SACTCHECK · CHAPTER 04

# Your record and contact card

Nivolumab, gemcitabine and carboplatin before surgery · National Cancer Control Programme (NCCP) 00849

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

Record only the medicines given at each visit. Day 8 is gemcitabine alone. There are at most three cycles under this regimen.

**Neoadjuvant cycle 1 · Days 1 and 8** · copy this sheet for another cycle of the same phase.

## Nivolumab · Immunotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Breathing / cough** | □ Usual breathing | □ New cough or breathlessness: call promptly | □ Symptoms with usual activity: urgent review | □ Breathless at rest: urgent assessment | □ Severe breathing difficulty / collapse: 112/999 |
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/ day or at night: call | □ 7+ extra stools/ day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |

## Gemcitabine · Chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |
| **Nausea** | □ None | □ Queasy but eating and drinking | □ Intake reduced: contact the team | □ Unable to maintain intake: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Mouth soreness / ulcers** | □ None | □ Mild soreness; eating normally | □ Painful; soft food needed: call | □ Cannot eat / drink enough: urgent review | □ Major breathing difficulty: 112/999 |

## Carboplatin · Chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Numbness / tingling** | □ None | □ Mild change; usual tasks unaffected | □ Limits buttons, writing or walking: call | □ Washing, dressing or walking difficult: urgent review | □ Sudden major weakness / paralysis: emergency help |
| **Bleeding / bruising** | □ None | □ New small bruise / brief bleed: report | □ Repeated bleeding: call today | □ Bleeding that will not stop: urgent help | □ Heavy bleeding / collapse: 112/999 |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Cycle number: _____ Start date: _____ Next review: _____

Record only the medicines given at each visit. Day 8 is gemcitabine alone. There are at most three cycles under this regimen.

**Neoadjuvant cycle 2 · Days 1 and 8** · copy this sheet for another cycle of the same phase.

## Nivolumab · Immunotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Breathing / cough** | □ Usual breathing | □ New cough or breathlessness: call promptly | □ Symptoms with usual activity: urgent review | □ Breathless at rest: urgent assessment | □ Severe breathing difficulty / collapse: 112/999 |
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/ day or at night: call | □ 7+ extra stools/ day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |

## Gemcitabine · Chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |
| **Nausea** | □ None | □ Queasy but eating and drinking | □ Intake reduced: contact the team | □ Unable to maintain intake: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Mouth soreness / ulcers** | □ None | □ Mild soreness; eating normally | □ Painful; soft food needed: call | □ Cannot eat / drink enough: urgent review | □ Major breathing difficulty: 112/999 |

## Carboplatin · Chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Numbness / tingling** | □ None | □ Mild change; usual tasks unaffected | □ Limits buttons, writing or walking: call | □ Washing, dressing or walking difficult: urgent review | □ Sudden major weakness / paralysis: emergency help |
| **Bleeding / bruising** | □ None | □ New small bruise / brief bleed: report | □ Repeated bleeding: call today | □ Bleeding that will not stop: urgent help | □ Heavy bleeding / collapse: 112/999 |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |

Dates / doses missed or changed / symptoms affecting daily life / questions:

> Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Cycle number: _____ Start date: _____ Next review: _____

Record only the medicines given at each visit. Day 8 is gemcitabine alone. There are at most three cycles under this regimen.

**Neoadjuvant cycle 3 · Days 1 and 8** · copy this sheet for another cycle of the same phase.

### Nivolumab · Immunotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Breathing / cough** | □ Usual breathing | □ New cough or breathlessness: call promptly | □ Symptoms with usual activity: urgent review | □ Breathless at rest: urgent assessment | □ Severe breathing difficulty / collapse: 112/999 |
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/ day or at night: call | □ 7+ extra stools/ day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |

### Gemcitabine · Chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |
| **Nausea** | □ None | □ Queasy but eating and drinking | □ Intake reduced: contact the team | □ Unable to maintain intake: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Mouth soreness / ulcers** | □ None | □ Mild soreness; eating normally | □ Painful; soft food needed: call | □ Cannot eat / drink enough: urgent review | □ Major breathing difficulty: 112/999 |

### Carboplatin · Chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Numbness / tingling** | □ None | □ Mild change; usual tasks unaffected | □ Limits buttons, writing or walking: call | □ Washing, dressing or walking difficult: urgent review | □ Sudden major weakness / paralysis: emergency help |
| **Bleeding / bruising** | □ None | □ New small bruise / brief bleed: report | □ Repeated bleeding: call today | □ Bleeding that will not stop: urgent help | □ Heavy bleeding / collapse: 112/999 |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |

Dates / doses missed or changed / symptoms affecting daily life / questions:

> Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

# Carry your treatment details

Print, cut out and fold. Follow the contact plan from your own hospital.

SACTCHECK · SACT TREATMENT

## Nivolumab, gemcitabine and carboplatin before surgery

NCCP 00849 v2 · 21-day cycle

Up to three 21-day cycles. Day 1: nivolumab, gemcitabine and carboplatin. Day 8: gemcitabine alone after blood tests and review. Then surgical MDT reassessment. The five listed backbones are alternatives; this guide covers only your selected combination.

| 1 | 2 | 3 | 4 | 5 | 6 |
|---|---|---|---|---|---|

**Your contact plan**

Oncology team: Insert numbers here
Out of hours: Insert numbers here.

Evenings / overnight: instructed out-of-hours pathway or nearest ED.
**Emergency: 112 / 999.**

Other hospital / own contact: _____

QR: https://sactcheck.com/patient/00849/backbone_00310/ · no patient information.

Draft for review · SACTCheck v0.84.27

# Sources and further information

Nivolumab, gemcitabine and carboplatin before surgery · National Cancer Control Programme (NCCP) 00849

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.


1. **NCCP 00849 v2 · pNCCP 00849 pp1–7; NCCP 00310 supportive care; NCIS V8 Table 1**
   https://healthservice.hse.ie/documents/6865/849_V2_Nivolumab_chemotherapy.pdf
2. **NCCP 00310 v8 · selected chemotherapy**
   https://healthservice.hse.ie/documents/6713/310_GemCARBOAUC_5.pdf
3. **EMA Opdivo product information**
   https://www.ema.europa.eu/en/documents/product-information/opdivo-epar-product-information_en.pdf
4. **CheckMate 816 final overall survival**
   https://www.nejm.org/doi/10.1056/NEJMoa2502931
5. **NCCP NCIS antiemetics V8**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf